



Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HWAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Nodes		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials: <i>lw</i>	Date Reviewed: <i>1/10/12</i>	

***** MODIFIED REPORT - REVIEW ADDENDUM SECTION *****

DIAGNOSIS

(A) LEFT BREAST, SEGMENTAL MASTECTOMY:

INVASIVE LOBULAR CARCINOMA, CLASSICAL TYPE, MODIFIED BLACK'S NUCLEAR GRADES 1 AND 2.

INVASIVE CARCINOMA COVERS AN AREA OF APPROXIMATELY 3 CM..

INVASIVE CARCINOMA EXTENDS TO WITHIN 3.0 MM OF DEEP, 3.0 MM OF INFERIOR AND 5.0 MM OF SUPERFICIAL MARGINS.

LYMPHOVASCULAR INVASION IDENTIFIED.

LOBULAR CARCINOMA IN SITU.

Intraductal papilloma.

Fibrocystic changes including duct epithelial hyperplasia without atypia, apocrine metaplasia, radial scar and cyst.

Microcalcifications present and associated with fibrocystic changes.

(B) LEFT BREAST CAPSULE, CAPSULECTOMY:

Dense fibrous connective tissue, no tumor present.

(C) LEFT BREAST, NEW SUPERFICIAL INFERIOR MARGIN, EXCISION:

Breast parenchyma with fibrocystic changes including florid ductal hyperplasia without atypia, apocrine metaplasia and microcysts.

No evidence of malignancy.

(D) LEFT BREAST, ADDITIONAL SUPERFICIAL MARGIN, EXCISION:

Skin and subcutaneous adipose tissue, no evidence of malignancy.

(E) LEFT AXILLA, LEVEL 1 AND 2 LYMPH NODES, EXCISION:

METASTATIC CARCINOMA IN ONE OF TWELVE LYMPH NODES.

METASTATIC CARCINOMA MEASURES 2.8 CM IN GREATEST DIMENSION.

EXTRANODAL EXTENSION UP TO 3.0 MM PRESENT.

(F) LEFT AXILLARY LEVEL 3 LYMPH NODE:

One lymph node, no tumor present.

1CD-0-3
carcinoma, infiltrating lobular, NOS 8520/3
Site: breast, NOS 050.9
lw
1/11/12

GROSS DESCRIPTION

(A) LEFT SEGMENTAL MASTECTOMY, SHORT SUPERIOR, LONG LATERAL, PURPLE INK DEEP - One pale-yellow segmental mastectomy specimen (9.5 x 8.0 x 4.2 cm) with a short stitch to mark the superior, long stitch to mark lateral and purple ink to mark the deep. The specimen is inked, serially sectioned from medial to lateral into twelve slices. Located at slice 3 through slice 8, there is one pale-gray, firm, ill-defined nodule (3.2 x 2.1 x 2.7 cm) is identified, located at 0.2 cm from deep margin, 0.2 cm from the anterior margin, 0.2 cm from superior margin, 1.2 cm from inferior margin, 0.6 cm from medial margin and 2.2 cm from lateral margin. One clip is identified at slice 5, central portion. A portion of normal tissue and tumor is submitted for tumor bank.

INK CODE: Blue - superior; green - inferior; yellow - superficial; black - deep; red - lateral and medial.

SECTION CODE: A1, slice 1, entirely submitted of the slice, perpendicular for medial margin; A2, slice 12, central portion lateral margin perpendicular; A3, slice 2, central portion with deep margin section adjacent to the lesion at slice 3; A4, slice 3, central portion nodule with anterior margin; A5, slice 3, central portion nodule with deep margin; A6, slice 3, central to inferior portion with the anterior margin; A7, slice 3 central to inferior portion tumor with the deep margin; A8-A14, entirely submitted of the slice 4 from superior to inferior direction; A15-A20, entirely submitted of the slice 5 from superior to inferior direction (A16 associated with clip); A21-A28, slice 6, entirely submitted of the slice submitted from superior to inferior direction; A29, slice 7, inferior portion with inferior and deep margin; A30, slice 8, superior portion with the superior margin; A31, A32, slice 8, central portion nodule with the deep nodule submitted from superior to inferior direction; A33, slice 9, central portion section adjacent to the lesion at slice 8.

*GROSS EVALUATION: TUMOR EXTENDS SUPERFICIAL/SUPERIOR AND DEEP MARGINS.

(B) LEFT BREAST IMPLANT AND PARTIAL CAPSULECTOMY - An 11.5 x 11.5 x 2.5 cm tan-yellow plastic breast implant devoid of contents. There is a 7.0 x 2.5 x 1.0 cm irregular piece of tan-yellow fibroadipose tissue attached to the surface of the breast implant. No inscriptions are noted. Representative sections of the fibroadipose tissue is submitted in B1-B2.

Photos are also taken.

(C) NEW SUPERFICIAL INFERIOR MARGIN, INK MARKS TRUE MARGIN - One pale-yellow fibroadipose tissue (4.5 x 1.8 x 0.6 cm) with one side inked with purple to mark the true margin. The true margin is reinked and the specimen is serially sectioned and entirely submitted.

INK CODE: Blue - true margin.

SECTION CODE: C1-C4, entirely submitted of the specimen.

(D) ADDITIONAL SUPERFICIAL MARGIN, INK ON TRUE MARGIN - One pale-yellow fibroadipose tissue (2.5 x 1.6 x 0.6 cm) with purple ink to mark the true margin. The true margin is inked and the specimen is serially sectioned and entirely submitted. Also in the container, there are two unremarkable pale-gray skin strips (5.2 x 0.5 x 0.5 cm, 3.6 x 0.4 x 0.6 cm, respectively). The resection margin of the skin is inked and specimen is serially sectioned and entirely submitted.

INK CODE: Blue - resection margin.

SECTION CODE: D1, entirely submitted of the fibroadipose tissue; D2, D3, each containing one piece of skin strip.

(E) LEFT AXILLARY, LEVEL 1 AND 2 LYMPH NODES - An 11.0 x 7.5 x 2.5 cm portion of yellow fibroadipose tissue which yields multiple possible lymph nodes ranging from 0.5 cm to 4.0 x 3.2 x 1.5 cm. The largest lymph node has a solid tan-white lobulated cut surface suggestive of metastases. The lymph nodes are submitted entirely.

SECTION CODE: E1-E4, one lymph node bisected in each cassette; E5, two lymph nodes; E6, four lymph nodes; E7, largest lymph node.

(F) LEFT AXILLARY LEVEL 3 LYMPH NODE - A 3.0 x 2.5 x 0.8 cm portion of yellow fibroadipose tissue which yields two possible lymph nodes, both measuring 1.0 cm in greatest dimension. The lymph nodes are submitted entirely in F.

CLINICAL HISTORY

None given.

SNOMED CODES

T-04050, M-85203, T-C4710, M-85206

"Some tests reported here may have been developed and performance characteristics determined by specifically cleared or approved by the U.S. Food and Drug Administration."

Entire report and diagnosis completed by:

.. These tests have not been

Start of ADDENDUM

This report is issued to give Immunohistochemistry results. Immunohistochemical staining is performed on a representative Pen-Fixed, paraffin-embedded section of METASTATIC CARCINOMA, axillary lymph node, block E7.

MARKER	Clone/Vendor	RESULTS	STAINING	SCORE
HER2 overexpression	AB8 (NeoMarkers)	Negative		0

FOOTNOTE

Breast specimens used for determining prognostic / predictive markers are fixed in either formalin or Pen-Fix (buffered alcoholic formalin fixative) for 6 - 48 hours.

For ER & PR:	Positive	10 - 100%
	Low Positive	1-9%

For Ki-67	Low Positive	<17%
	Intermediate Positive	17-35%
	High Positive	>35%

Entire report and diagnosis completed by:

-----END OF REPORT-----